

INTERNATIONAL A-LEVEL PSYCHOLOGY

PS03

Unit 3 Advanced Topics and Research Methods 2

Mark scheme

January 2025

Version: 1.0 Final



Mark schemes are prepared by the Lead Assessment Writer and considered, together with the relevant questions, by a panel of subject teachers. This mark scheme includes any amendments made at the standardisation events which all associates participate in and is the scheme which was used by them in this examination. The standardisation process ensures that the mark scheme covers the students' responses to questions and that every associate understands and applies it in the same correct way. As preparation for standardisation each associate analyses a number of students' scripts. Alternative answers not already covered by the mark scheme are discussed and legislated for. If, after the standardisation process, associates encounter unusual answers which have not been raised they are required to refer these to the Lead Examiner.

It must be stressed that a mark scheme is a working document, in many cases further developed and expanded on the basis of students' reactions to a particular paper. Assumptions about future mark schemes on the basis of one year's document should be avoided; whilst the guiding principles of assessment remain constant, details will change, depending on the content of a particular examination paper.

Further copies of this mark scheme are available from [oxfordaqaexams.org.uk](https://www.oxfordaqaexams.org.uk)

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Level of response marking instructions

Level of response mark schemes are broken down into levels, each of which has a descriptor. The descriptor for the level shows the average performance for the level. There are marks in each level.

Before you apply the mark scheme to a student's answer read through the answer and annotate it (as instructed) to show the qualities that are being looked for. You can then apply the mark scheme.

Step 1 Determine a level

Start at the lowest level of the mark scheme and use it as a ladder to see whether the answer meets the descriptor for that level. The descriptor for the level indicates the different qualities that might be seen in the student's answer for that level. If it meets the lowest level then go to the next one and decide if it meets this level, and so on, until you have a match between the level descriptor and the answer. With practice and familiarity you will find that for better answers you will be able to quickly skip through the lower levels of the mark scheme.

When assigning a level you should look at the overall quality of the answer and not look to pick holes in small and specific parts of the answer where the student has not performed quite as well as the rest. If the answer covers different aspects of different levels of the mark scheme you should use a best fit approach for defining the level and then use the variability of the response to help decide the mark within the level, ie if the response is predominantly level 3 with a small amount of level 4 material it would be placed in level 3 but be awarded a mark near the top of the level because of the level 4 content.

Step 2 Determine a mark

Once you have assigned a level you need to decide on the mark. The descriptors on how to allocate marks can help with this. The exemplar materials used during standardisation will help. There will be an answer in the standardising materials which will correspond with each level of the mark scheme. This answer will have been awarded a mark by the Lead Examiner. You can compare the student's answer with the example to determine if it is the same standard, better or worse than the example. You can then use this to allocate a mark for the answer based on the Lead Examiner's mark on the example.

You may well need to read back through the answer as you apply the mark scheme to clarify points and assure yourself that the level and the mark are appropriate.

Indicative content in the mark scheme is provided as a guide for examiners. It is not intended to be exhaustive and you must credit other valid points. Students do not have to cover all of the points mentioned in the Indicative content to reach the highest level of the mark scheme.

An answer which contains nothing of relevance to the question must be awarded no marks.

Section A: Psychology of Sleep

Total for this section: 30 marks

Question	Marking Guidance	Total Marks
01	Which one of the following is a feature of non-REM sleep? Answer key: D – Synchronised EEG pattern	1 AO1 = 1

Question	Marking Guidance	Total Marks
02	What is meant by an infradian rhythm? Award 1 mark for an accurate definition of an infradian rhythm. A biological rhythm that occurs less than once every 24 hours. Accept alternative wording, eg lasts longer than 24 hours.	1 AO1 = 1

Question	Marking Guidance	Total Marks															
03	Evaluate research studies of circadian rhythms. Possible evaluation • Use of research studies to support existence of circadian rhythms, eg Siffre (1962, 1972 & 1999); Aschoff & Wever (1976). • Use of animal studies to support influence of SCN on circadian rhythms, eg Morgan (1995); DeCoursey et al. (2000). • Methodological issues with research studies, eg Siffre's study involved just one male participant and took place in an underground cave so the results cannot be generalised which might tell us little about overall natural human circadian rhythms; possibility of extraneous variables. • Issues with the use of animals in research and lack of generalisability to humans. • Applications of research in terms of understanding and minimising effects of jet lag and shift work, eg an understanding of circadian rhythms can help people working shifts to cope better with the disruption which will improve health; can also lead to better drug treatments if we understand when drugs have the optimum effects on the body. Credit other relevant material. <table border="1"> <thead> <tr> <th>Level</th><th>Description</th><th>Marks</th></tr> </thead> <tbody> <tr> <td>3</td><td>Evaluation of research studies into circadian rhythms is detailed and effective. The answer is clear with appropriate use of specialist terminology.</td><td>5–6</td></tr> <tr> <td>2</td><td>Evaluation of research studies into circadian rhythms is relevant but effectiveness is lacking. The answer lacks clarity in places. Specialist terminology is occasionally used appropriately.</td><td>3–4</td></tr> <tr> <td>1</td><td>Evaluation of research studies into circadian rhythms is very limited. The answer is vague/muddled. Specialist terminology is either absent or inappropriately used.</td><td>1–2</td></tr> <tr> <td>0</td><td>No creditable content.</td><td>0</td></tr> </tbody> </table>	Level	Description	Marks	3	Evaluation of research studies into circadian rhythms is detailed and effective. The answer is clear with appropriate use of specialist terminology.	5–6	2	Evaluation of research studies into circadian rhythms is relevant but effectiveness is lacking. The answer lacks clarity in places. Specialist terminology is occasionally used appropriately.	3–4	1	Evaluation of research studies into circadian rhythms is very limited. The answer is vague/muddled. Specialist terminology is either absent or inappropriately used.	1–2	0	No creditable content.	0	6 AO3 = 6
Level	Description	Marks															
3	Evaluation of research studies into circadian rhythms is detailed and effective. The answer is clear with appropriate use of specialist terminology.	5–6															
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0	No creditable content.	0															

Question	Marking Guidance	Total Marks									
04	Rashid is talking to his friend and says: “Sleep is really strange. Every night I lie down in my bed in a dark room and the next thing that happens is I wake up in the morning when it is light outside.” Describe the effects of endogenous pacemakers and exogenous zeitgebers on the sleep/wake cycle. Use Rashid’s statement to support your description. Possible content • Endogenous pacemakers are internal mechanisms that control biological rhythms. • Exogenous zeitgebers are external factors that influence/synchronise endogenous pacemakers. • Description of the role of the suprachiasmatic nucleus (SCN), pineal gland and the release of melatonin to control the sleep/wake cycle and how light influences this process. Possible application • When Rashid lies in bed in a dark room his eyes detect that there is no light. • These signals travel to Rashid’s SCN which stimulates his pituitary gland to release melatonin into his blood stream. • Melatonin slows his body and brain activity down which results in Rashid falling asleep. • In the morning, when his eyes detect the presence of light this sends a signal to his SCN which, in turn, signals the pineal gland to stop releasing melatonin and Rashid wakes up. Credit other relevant material. <table border="1" data-bbox="284 1422 1294 1933"> <thead> <tr> <th>Level</th><th>Description</th><th>Marks</th></tr> </thead> <tbody> <tr> <td>3</td><td>Knowledge of the effect of endogenous pacemakers and exogenous zeitgebers on the sleep/wake cycle and reference to Rashid’s statement is detailed and appropriate. Application is effective. The answer is clear with appropriate use of specialist terminology.</td><td>7–9</td></tr> <tr> <td>2</td><td>Knowledge of the effect of endogenous pacemakers and exogenous zeitgebers on the sleep/wake cycle and/or reference to Rashid’s statement is present. Application is mostly effective. The answer lacks clarity in places. Specialist terminology is occasionally used appropriately.</td><td>4–6</td></tr> </tbody> </table>	Level	Description	Marks	3	Knowledge of the effect of endogenous pacemakers and exogenous zeitgebers on the sleep/wake cycle and reference to Rashid’s statement is detailed and appropriate. Application is effective. The answer is clear with appropriate use of specialist terminology.	7–9	2	Knowledge of the effect of endogenous pacemakers and exogenous zeitgebers on the sleep/wake cycle and/or reference to Rashid’s statement is present. Application is mostly effective. The answer lacks clarity in places. Specialist terminology is occasionally used appropriately.	4–6	9 AO1 = 4 AO2 = 5
Level	Description	Marks									
3	Knowledge of the effect of endogenous pacemakers and exogenous zeitgebers on the sleep/wake cycle and reference to Rashid’s statement is detailed and appropriate. Application is effective. The answer is clear with appropriate use of specialist terminology.	7–9									
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	1	Knowledge of the effect of endogenous pacemakers and exogenous zeitgebers on the sleep/wake cycle and/or reference to Rashid's statement is very limited. Application is limited, poorly focused or absent. The answer is vague/muddled. Specialist terminology is either absent or inappropriately used.	1–3	
	0	No creditable content.	0	

Question	Marking Guidance	Total Marks
05	Briefly describe one evolutionary explanation for the function of sleep. Award marks as follows, 3 marks, the description of one evolutionary explanation of sleep is detailed. The answer is clear with appropriate use of specialist terminology. 2 marks, the description of one evolutionary explanation of sleep lacks detail. The answer lacks clarity in places. 1 mark, the description of one evolutionary explanation of sleep is briefly presented. The answer is very limited/vague/ muddled. Possible content • Hibernation theory: Webb proposed sleep serves a similar purpose to hibernation, eg sleep helps to conserve energy as when an animal is inactive, it expends less energy. • Diet: herbivores eat nutrient-poor vegetation so need to eat more to satisfy their energy requirements. Consequently, they are likely to spend more time foraging and less time asleep. Carnivores eat nutrient-rich meat so they do not need to eat a lot and can afford to sleep more. • Predator-prey status: predators can sleep longer due to no risk of predation. Some prey animals sleep for a long period of time if they sleep in a relatively safe place, increasing their survival chance as the risk of predation is low. However, other prey animals sleep for a short period of time as they sleep in a relatively unsafe place, increasing their survival chance as the risk of predation is high. • Risk of injury: sleep can help animals stay alive, as the risk of injury when awake is generally higher. Credit other relevant material.	3 AO1 = 3

Question	Marking Guidance	Total Marks
06	Explain one strength of the restoration theory of sleep. Award marks as follows, 3 marks, the strength of the restoration theory of sleep is detailed. The answer is clear with appropriate use of specialist terminology. 2 marks, the strength of the restoration theory of sleep lacks detail. The answer lacks clarity in places. 1 mark, the strength of the restoration theory of sleep is briefly presented. The answer is very limited/vague/muddled. Possible strengths • Use of evidence to support restoration theories, eg case studies of sleep deprivation show mental disturbances including hallucination and language disorders such as Shapiro (1981); Gardner (1964); Tripp (1960s). • Restoration theories are consistent with the high levels of REM sleep seen in newborns as their brains are developing at a quick rate and should, therefore, require more REM sleep. • Evidence from the effects of fatal familial insomnia, a rare inherited condition where people stop sleeping in middle age and usually die within two years. • REM rebound after deprivation supports the restorative role of REM sleep. Credit other relevant strengths.	3 AO3 = 3

Question	Marking Guidance	Total Marks
07	Briefly describe one explanation for narcolepsy. Award marks as follows, 3 marks, the description of one explanation of narcolepsy is detailed. The answer is clear with appropriate use of specialist terminology. 2 marks, the description of one explanation of narcolepsy lacks detail. The answer lacks clarity in places. 1 mark, the description of one explanation of narcolepsy is briefly presented. The answer is very limited/vague/muddled. Possible content • Thought to be linked to a genetic defect (seen in dogs – Lin et al (1999)) on chromosome 12. This is responsible for regulating brain receptors for a neurochemical called orexin/hypocretin. • Low levels of orexin/hypocretin (or its receptors) in the brain is thought to be the cause of narcolepsy. • Orexin/hypocretin plays an important role in maintaining wakefulness so a reduction in levels (or its receptors) is thought to be the reason why people with narcolepsy fall asleep uncontrollably. Credit other relevant material.	3 AO1 = 3

Question	Marking Guidance	Total Marks												
08	Briefly evaluate the genetic explanation for insomnia. Possible evaluation • Use of supporting evidence for genes playing a role in insomnia (chronotypes), eg Kerkhof and Van Dongen (1996), Van Dongen et al (2005). • Research evidence from family studies suggesting that insomnia runs in families, eg Ohayon et al. (2001). • Twin studies have shown evidence for a genetic component to insomnia, eg Bastian and Morin (2000), Watson et al. (2006). • Insomnia does not always run in families; many people can suffer from insomnia without a family history of the disorder. • Issues with twin study evidence as concordance between monozygotic (MZ) twins is never 100% when it should be if the cause is genetic as they share exactly the same genes. • Oversimplistic as it reduces the complex behaviour of insomnia down to just genetics, there is lots of research evidence to suggest that environmental factors also play a significant role, eg Riemann et al. (2010). Credit other relevant material. <table border="1"> <thead> <tr> <th>Level</th><th>Description</th><th>Marks</th></tr> </thead> <tbody> <tr> <td>2</td><td>Evaluation of the genetic explanation for insomnia is accurate with some detail. The answer is clear with appropriate use of specialist terminology.</td><td>3–4</td></tr> <tr> <td>1</td><td>Evaluation of the genetic explanation for insomnia is limited, vague or muddled. Specialist terminology is either absent or inappropriately used.</td><td>1–2</td></tr> <tr> <td>0</td><td>No creditable content.</td><td>0</td></tr> </tbody> </table>	Level	Description	Marks	2	Evaluation of the genetic explanation for insomnia is accurate with some detail. The answer is clear with appropriate use of specialist terminology.	3–4	1	Evaluation of the genetic explanation for insomnia is limited, vague or muddled. Specialist terminology is either absent or inappropriately used.	1–2	0	No creditable content.	0	4 AO3 = 4
Level	Description	Marks												
2	Evaluation of the genetic explanation for insomnia is accurate with some detail. The answer is clear with appropriate use of specialist terminology.	3–4												
1	Evaluation of the genetic explanation for insomnia is limited, vague or muddled. Specialist terminology is either absent or inappropriately used.	1–2												
0	No creditable content.	0												

Section B: Schizophrenia

Total for this section: 30 marks

Question	Marking Guidance	Total Marks
09	Mei has recently told her friend that she has been hearing voices. The voices comment on her daily activities and criticise her, telling her that she is a horrible person. Mei also told her friend that she has lost interest in going to school and just wants to stay at home. Mei's friend has noticed that Mei now rarely talks and when she is asked a question, her answers are very brief. Identify two symptoms of schizophrenia and explain how each symptom is demonstrated by Mei. For each symptom of schizophrenia, award marks as follows: 1 mark for accurately identifying a symptom of schizophrenia. 1 mark for a brief explanation of how Mei demonstrates that symptom. Possible content • (Auditory) hallucinations – Mei is hearing voices commenting on her activities/criticising her saying she is a horrible person. • Avolition – Mei has lost interest in day-to-day activities such as going to school (just wants to stay at home). • Speech poverty – Mei's friend has noticed that Mei has stopped talking/gives brief answers if asked a question. Note: 1 mark can be awarded for other accurate symptoms of schizophrenia, eg catatonic behaviour, however the additional mark cannot be achieved as Mei does not demonstrate this symptom.	4 AO1 = 2 AO2 = 2

Question	Marking Guidance	Total Marks												
10	Mei is diagnosed with schizophrenia by her doctor who refers her for cognitive therapy. Explain how cognitive therapy might be used to treat Mei. Possible content • The therapy could help Mei to identify the irrational thoughts/beliefs (eg that she is horrible). • The therapy might help Mei to understand the link between her thoughts (she is horrible), behaviour (not going to school) and emotions (loss of interest/apathy). • The therapy might help Mei understand that the voices she hears are not real and that they might just be her own inner voice/thoughts. Credit other relevant material. A maximum of 1 mark if there is no application to Mei. <table border="1"> <thead> <tr> <th>Level</th><th>Description</th><th>Marks</th></tr> </thead> <tbody> <tr> <td>2</td><td>Knowledge of cognitive therapy as a treatment for Mei is accurate with some detail. The answer is clear with appropriate use of specialist terminology.</td><td>3–4</td></tr> <tr> <td>1</td><td>Knowledge of cognitive therapy as a treatment for Mei is limited, vague or muddled. Specialist terminology is either absent or inappropriately used.</td><td>1–2</td></tr> <tr> <td>0</td><td>No creditable content.</td><td>0</td></tr> </tbody> </table>	Level	Description	Marks	2	Knowledge of cognitive therapy as a treatment for Mei is accurate with some detail. The answer is clear with appropriate use of specialist terminology.	3–4	1	Knowledge of cognitive therapy as a treatment for Mei is limited, vague or muddled. Specialist terminology is either absent or inappropriately used.	1–2	0	No creditable content.	0	4 AO2 = 4
Level	Description	Marks												
2	Knowledge of cognitive therapy as a treatment for Mei is accurate with some detail. The answer is clear with appropriate use of specialist terminology.	3–4												
1	Knowledge of cognitive therapy as a treatment for Mei is limited, vague or muddled. Specialist terminology is either absent or inappropriately used.	1–2												
0	No creditable content.	0												

Question	Marking Guidance	Total Marks
11	Briefly explain one strength of cognitive therapy as a treatment for schizophrenia. Award marks as follows: 2 marks for a clear explanation of a strength of cognitive therapy. 1 mark for a muddled or vague strength of cognitive therapy. Possible strengths • Use of supporting research for cognitive therapy, eg Drury et al. (1996) reduction in positive symptoms and recovery time; Kuipers et al. (1997) higher levels of patient satisfaction and engagement; Morrison et al. (2014) those receiving cognitive therapy had positive clinical outcomes and better functioning than those who did not; Morrison et al. (2012) cognitive therapy reduced the severity of psychotic episodes. • Cognitive therapies put the patient in charge of their recovery, unlike other treatments. • No issues with side effects, unlike using antipsychotics. Credit other relevant strengths.	2 AO3 = 2

Question	Marking Guidance	Total Marks
12	Discuss issues of reliability and validity in the diagnosis of schizophrenia. Possible content • Validity in diagnosis refers to the accuracy of a diagnosis. In the case of schizophrenia, whether someone actually has schizophrenia, or whether they have a different disorder. • Reliability in diagnosis refers to the consistency of a diagnosis. In the case of schizophrenia, whether different experts/doctors would all agree that an individual has schizophrenia. • Reliability is linked to validity. If a diagnosis of schizophrenia is not reliable then it cannot be valid. • Reliability can be assessed using test-retest, when a clinician makes the same diagnosis of the same person on separate occasions from the same information; and inter-rater reliability, when different clinicians make the same diagnosis of the same person. • Diagnosis of schizophrenia can change over time. The diagnostic criteria in the ICD/DSM change as they are updated which can affect both the reliability and validity of a diagnosis. • Knowledge of issues such as cultural bias, comorbidity, culture and gender bias is relevant provided these are related to the reliability/validity of the diagnosis of schizophrenia. Possible discussion • Supporting research for reliability, eg as each edition of the DSM has been published inter-rater reliability has improved - Beck et al (1962) found a 54% concordance rate between practitioners whereas Soderberg et al. (2005) found a concordance rate of 81%; Jakobson et al (2005). • Contradictory research for reliability, eg Rosenhan (1973), Read et al. (2004). • Use of supporting research for validity, eg Hollis (2000), Jager et al. (2003). • Use of contradictory research for validity, eg Baillie et al. (2009), Allardyce et al. (2006). • People diagnosed with schizophrenia can differ greatly with respect to symptoms, which suggests the concept of a single label of schizophrenia is not valid and perhaps the diagnosis as 'schizophrenia' should not be used. • Description of possible issues that might affect reliable/valid diagnosis of schizophrenia eg comorbidity, symptom overlap, cultural and gender biases. Credit other relevant material.	20 AO1 = 8 AO3 = 12

Level	Description	Marks
4	Discussion of the issues of reliability and validity in the diagnosis of schizophrenia is mostly effective. Minor detail and/or expansion of argument is sometimes lacking. The answer is clear, organised and focused. Specialist terminology is used effectively.	16–20
3	There is some effective discussion of the issues of reliability and/or validity in the diagnosis of schizophrenia. The answer is mostly clear and organised but occasionally lacks focus. Specialist terminology is used appropriately. OR just reliability or validity discussed at level 4.	11–15
2	Discussion of the issues of reliability and/or validity in the diagnosis of schizophrenia is of limited effectiveness. The answer lacks clarity, accuracy and organisation in places. Specialist terminology is occasionally used appropriately. OR just reliability or validity discussed at level 3.	6–10
1	Discussion of the issues of reliability and/or validity in the diagnosis of schizophrenia is limited and poorly focused. The answer as a whole lacks clarity, has many inaccuracies and is poorly organised. Specialist terminology is either absent or inappropriately used. OR just reliability or validity discussed at level 2.	1–5
0	No creditable content.	0

Section C: Research Methods 2

Total for this section: 30 marks

	A psychologist carried out a study of depression. 18 people who had been diagnosed with depression participated in the study. The psychologist asked the participants to keep a daily record of their main thoughts, emotions and behaviours in a diary for two weeks. At the end of the two weeks, the psychologist collected the participants' diaries and conducted a content analysis on the information written in them.	
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Question	Marking Guidance	Total Marks
13	What is meant by content analysis? Award marks as follows 2 marks for a clear definition of content analysis. 1 mark for a muddled or vague definition of content analysis. Content A research technique that involves coding written/qualitative information received about behaviour into themes/concepts so that these can be quantified and analysed. Credit alternative wording.	2 AO1 = 2

Question	Marking Guidance	Total Marks															
14	Use your knowledge of how to conduct a content analysis of written information to explain what the psychologist could do with the information in the participants' diaries. Possible content • The psychologist should carefully read through each of the participants' diaries to gain an idea of their content. • The psychologist should develop a coding system that they can use to analyse the diary entries, eg they could decide on certain words to look out for as they re-read the diaries, such as sad/unhappy, unmotivated. • The psychologist should then re-read the diaries and when they see something that relates to their coding system they should record it with a tally mark. • (After reading all 18 diaries), the psychologist should count the total number of tallies per coded category and interpret the results. • The psychologist might also carry out a thematic analysis where they categorise the findings into overall themes and concepts. Credit other relevant material. <table border="1"> <thead> <tr> <th>Level</th><th>Description</th><th>Marks</th></tr> </thead> <tbody> <tr> <td>3</td><td>Application of how the psychologist could conduct the content analysis is detailed and appropriate. The answer is clear with appropriate use of specialist terminology.</td><td>5–6</td></tr> <tr> <td>2</td><td>Application of how the psychologist could conduct the content analysis is relevant but detail is lacking. The answer lacks clarity in places. Specialist terminology is occasionally used appropriately.</td><td>3–4</td></tr> <tr> <td>1</td><td>Application of how the psychologist could conduct the content analysis is very limited. The answer is vague/muddled. Specialist terminology is either absent or inappropriately used.</td><td>1–2</td></tr> <tr> <td>0</td><td>No creditable content.</td><td>0</td></tr> </tbody> </table>	Level	Description	Marks	3	Application of how the psychologist could conduct the content analysis is detailed and appropriate. The answer is clear with appropriate use of specialist terminology.	5–6	2	Application of how the psychologist could conduct the content analysis is relevant but detail is lacking. The answer lacks clarity in places. Specialist terminology is occasionally used appropriately.	3–4	1	Application of how the psychologist could conduct the content analysis is very limited. The answer is vague/muddled. Specialist terminology is either absent or inappropriately used.	1–2	0	No creditable content.	0	6 AO2 = 6
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3	Application of how the psychologist could conduct the content analysis is detailed and appropriate. The answer is clear with appropriate use of specialist terminology.	5–6															
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0	No creditable content.	0															

Question	Marking Guidance	Total Marks												
15	Explain how the psychologist could assess the reliability of the content analysis of the participants' diaries. Possible content - The psychologist could recruit (at least) one more independent researcher/observer and they agree on the coding to use. - Both the psychologist and the other observer(s) could independently read through all 18 participant's diaries using the pre-agreed coding. - The psychologist and observer(s) compare and statistically analyse their results, using a correlation, to check for the level of agreement between them. - If a significant positive correlation is found between the psychologist and observer's data, the results would be considered reliable. Credit other relevant material, such as test-retest. A maximum of 1 mark if there is no application to this study. <table border="1"> <thead> <tr> <th>Level</th><th>Description</th><th>Marks</th></tr> </thead> <tbody> <tr> <td>2</td><td>Application of how the psychologist could assess the reliability of the content analysis is relevant but detail is lacking. The answer lacks clarity in places.</td><td>3–4</td></tr> <tr> <td>1</td><td>Application of how the psychologist could assess the reliability of the content analysis is very limited. The answer is vague/muddled.</td><td>1–2</td></tr> <tr> <td>0</td><td>No creditable content.</td><td>0</td></tr> </tbody> </table>	Level	Description	Marks	2	Application of how the psychologist could assess the reliability of the content analysis is relevant but detail is lacking. The answer lacks clarity in places.	3–4	1	Application of how the psychologist could assess the reliability of the content analysis is very limited. The answer is vague/muddled.	1–2	0	No creditable content.	0	4 AO2 = 4
Level	Description	Marks												
2	Application of how the psychologist could assess the reliability of the content analysis is relevant but detail is lacking. The answer lacks clarity in places.	3–4												
1	Application of how the psychologist could assess the reliability of the content analysis is very limited. The answer is vague/muddled.	1–2												
0	No creditable content.	0												

Question	Marking Guidance	Total Marks
16	Only 6 out of the 18 people who participated in the study were male. Calculate what fraction of the participants was female. Show your answer in its simplest form. Award marks as follows 2 marks for the correct answer in its simplest form: 2/3rds of participants were female. 1 mark for a correct answer that has not been shown in its simplest form. 18–6 = 12 females 12/18 in its simplest form = 2/3	2 AO2 = 2

Question	Marking Guidance	Total Marks
17	The psychologist was concerned that the participants may have been affected by demand characteristics when writing their diary responses. Explain why the psychologist might be concerned about demand characteristics. Award marks as follows, 3 marks, the explanation of why the psychologist might be concerned about demand characteristics is detailed. The answer is clear with appropriate use of specialist terminology. 2 marks, the explanation of why the psychologist might be concerned about demand characteristics lacks detail. The answer lacks clarity in places. 1 mark, the explanation of why the psychologist might be concerned about demand characteristics is briefly presented. The answer is very limited/vague/muddled or an answer not explicitly applied to this study. Possible content • The participants may have under/overexaggerated or under/overemphasised how they were feeling during the two weeks, suggesting they felt significantly more/less unhappy than they actually were. • Most people know that depression is linked to thinking negatively about yourself. Therefore, in order to help the psychologist, the participants may have lied in the diary and stated they had experienced many examples of negative thoughts during the two weeks. Credit other relevant material.	3 AO2 = 3

Question	Marking Guidance	Total Marks
18	As part of the content analysis, the psychologist collected nominal data. What is meant by nominal data? Award marks as follows, 2 marks for a clear definition of nominal data. 1 mark for a muddled or vague definition of nominal data. Possible content • A basic form of measurement where frequency data is collected using categories, often by tallying. • The data is classed as discrete data, and it cannot be placed into any meaningful order. Credit alternative wording. Credit use of an example.	2 AO1 = 2

Question	Marking Guidance	Total Marks
19	Apart from nominal data, name and outline one other type of data that the psychologist might have collected during this study. Award marks as follows, 1 mark for naming a relevant data type. AND 2 marks for a clear outline of how the named data type could be collected in the study. 1 mark for a muddled or vague outline of how the named data type could be collected in the study. Possible content • Primary: the psychologist collected the data for their study first-hand by asking the participants to complete a diary for the 2-week period which was then analysed by the psychologist themselves. • Qualitative: the psychologist may have used a thematic analysis approach to identify an overall trend in any themes or concepts that were shared across all of the diaries. This would produce detailed, non-numerical data. Note: 1 mark can be awarded for naming other types of data that might have been collected in the study, eg ordinal, interval or qualitative. However, to gain the further 2 marks, the answer must clearly and explicitly explain how the psychologist might have collected that type of data within the study, eg for ordinal, suggesting that the psychologist might have asked participants to give a daily rating of their happiness on a scale of 1 to 10, where 1 was very unhappy and 10 was very happy, which would be a subjective rating scale of happiness.	3 AO2 = 3

Question	Marking Guidance	Total Marks
20	Explain one strength of conducting a content analysis. Award marks as follows, 3 marks, a strength of a content analysis is detailed. The answer is clear with appropriate use of specialist terminology. 2 marks, a strength of a content analysis lacks detail. The answer lacks clarity in places. 1 mark, a strength of a content analysis is briefly presented. The answer is very limited/vague/muddled. Possible strengths • Usually higher in ecological validity since content analysis use real-world content. • Content analyses are usually high in reliability as researchers can look back through the content to check the accuracy of their results. • They often collect both quantitative and qualitative data which allows researchers the benefits of using both types of data in their results and conclusions. • There are usually fewer ethical issues involved with content analysis as the medium that is analysed is often in the public domain or consented to/willingly provided (such as the diary entries in this study). • As content analysis, usually, involves little to no interaction with people then it is less likely that the researchers can influence the content directly (reduces investigator/researcher effects). Credit other relevant material.	3 AO3 = 3

Question	Marking Guidance	Total Marks
21	The psychologist wrote a scientific report of the content analysis. The introduction section referred to information taken from a book by Aaron Beck. Which of the following would be the correct way of writing the reference for this book in the References section of the report? Shade one box only. Answer key: A – Beck, A. (1967). <i>Depression: Clinical, Experimental and Theoretical Aspects</i>. New York: Hoeber.	1 AO2 = 1

Question	Marking Guidance	Total Marks
22	Meta-analysis is an additional research method that can be used in psychology. What is meant by meta-analysis? Award marks as follows, 2 marks for a clear definition of meta-analysis. 1 mark for a muddled or vague definition of meta-analysis. Possible content Combining the results of studies with similar research hypothesis and statistically analysing them to provide a single overall conclusion which has greater statistical power. Accept alternative wording.	2 AO1 = 2

Question	Marking Guidance	Total Marks
23	Briefly explain one limitation of meta-analysis. Award marks as follows, 2 marks for a clear limitation of meta-analysis. 1 mark for a muddled or vague limitation of meta-analysis. Possible content • Possibility of publication bias as the researcher can choose which studies to include within their meta-analysis and might only select studies that support their ideas/hypothesis. • Meta-analysis relies on secondary data, there is no guarantee that the original studies were of good quality, which reduces the overall validity of any conclusions drawn from the data. Credit other relevant material.	2 AO3 = 2

PS03 grid

	AO1	AO2	AO3	Total
Section A				
01	1			1
02	1			1
03			6	6
04	4	5		9
05	3			3
06			3	3
07	3			3
08			4	4
Section B				
09	2	2		4
10		4		4
11			2	2
12	8		12	20
Section C				
13	2			2
14		6		6
15		4		4
16		2		2
17		3		3
18	2			2
19		3		3
20			3	3
21		1		1
22	2			2
23			2	2
Unit total	28	30	32	90